

QUESTION bank

HYPERTENSIVE CRISIS

- 1- Which of the following Clinical manifestation of hypertensive urgency
 - a) Retinopathy
 - b) Papilledema
 - c) cough
 - d) Severe headache, nose bleeding**
- 2- Which of the following characterized of Hypertensive emergency
 - A) Sudden elevation of Bpr 180/110 or more
 - B) Sudden elevation of Bpr 180/110 with organ damage**
 - C) Elevation of Bpr 150/100 with organ damage
 - D) Elevation of Bpr 150/100 or more
- 3- Diagnostic test for hypertensive crisis include all of the following except
 - A) Measuring blood pressure from RT ,LT arm
 - B) ECG
 - C) chest radiograph
 - D) abdominal ultrasound**
- 4- Which of the following Causes of hypertensive crisis
 - A) Intracerebral hemorrhage**
 - B) Acute appendicitis
 - C) Pneumonia
 - D) Abdominal hernia
- 5- A patient with BP 190/120 mm hg without symptoms is classified as
 - A- Hypertensive emergency
 - B- Hypertensive urgency**
 - C- Stage 1 hypertensive
 - D- Secondary hypertension
- 6- Which condition is commonly associated with hypertensive emergency
 - A- Aortic dissection**
 - B- Asthma
 - C- Diabetes mellitus
 - D- Hypothyroidism

7- First line treatment in hypertensive emergency

A- Oral antihypertensive

B- Lifestyle modification

C- Intravenous antihypertensive

D- Diuretics only

8- Mention 4 point for patient education about control of Blood pressure

9- Mention 4 nursing diagnosis for patient with hypertensive crisis

10- Mention 4 nursing management to the patients with hypertensive urgency

DIC

1- Which of the following risk factors of DIC

A- Complications of pregnancy: abruptio placentae

B- Hypertensive emergency

C- Acute myocardial infarction

D- Acute lung injury

2- Which of the following include clinical manifestation of DIC

A- Bleeding, from many sites in the body

B- Blood clots

C- Bruising

D- All of the above

3- Which of the following diagnostic test for DIC

A- PT, PTT, INR

B- D. Diamer

C- A, B

D- CBC

4- Which of the following treatment of severe thrombocytopenia + bleeding

a- FFP

b- Platelet

c- Antifibrinolytic treatment

d- Heparin

- 5- The most common cause of DIC IS
A- Trauma
B- Sepsis
C- Hypertension
D- Asthma
- 6- Platelet count in DIC is usually
A- Normal
B- Increased
C- Decreased
D- Variable
- 7- Which of the following treatment of active bleeding in DIC
A- FFP
B- Platelet
C- Antifibrinolytic treatment
D- Heparin
- 8- Mention 4 nursing intervention for patients with DIC
- 9- Mention 4 institution percussion to patients with DIC to prevent bleeding
- 10- Mention 4 nursing intervention to Enhancing Tissue Perfusion IN DIC patients

Stroke

- 1- Which of the following disorder consider high incidence of stroke
a- Hemorrhagic stroke
b- Ischemic stroke
c- Brain tumor
d- Cerebral aneurism
- 2- Which of the following manifestation consider hall mark in hemorrhagic stroke
a- Numbness and weakness of affected side
b- Sever headache and disturbed level of consciousness
c- Visual defect
d- Ataxia

- 3- Which of the following consider Causes of hemorrhagic stroke
- a- Smoking
 - b- Hyperlipidemia
 - c- Arteriovenous malformation**
 - d- Heart disease
- 4- patients presents with sudden weakness on one side of body and difficulty speaking .the most likely diagnosis is
- a- migraine
 - b- Stroke**
 - c- Seizure
 - d- brain tumor
- 5- what is the accurate time for administration of IV thrombolysis (TPA) in acute ischemic stroke ?
- a- within 1 hour
 - b- within 3-4.5 hours**
 - c- within 12 hours
 - d- within 24 hours
- 6- which diagnostic test of the following is first used to suspected stroke
- a- MRI
 - b- CT scan**
 - c- X-ray
 - e- Ultrasound
- 7- What is a transient ischemic attack (TIA)
- A- Premature brain damage
 - B- Temporary blockage with reversable damage**
 - C- Brain hemorrhage
 - D- Infection of the brain
- 8- Mention 4 diagnostic test for stroke patients
- 9- Mention 4 nursing care to patients with acute stroke
- 10- Mention 4 nursing care to acute stroke patients during ?(TPA) administration

Myasthenia graves

1- Myasthenia graves described as

A- Neurodegenerative disorder

B- An autoimmune disease affecting neuromuscular junction

C- A muscular dystrophy

D- Ametabolic disorder

2- A classic symptom of myasthenia gravis is

A- Muscle rigidity

B- Muscle weakness that improves with activity

C- Muscle weakness that worse with activity

D- Tremors

3- Which of the following disorders are commonly associated with myasthenia graves

A- Thyroid tumor

B- Thymus gland disorder

C- Pituitary adenoma

D- Adrenal tumor

4- Which test is used to diagnose myasthenia graves

A- ECG

B- EMG

C- EEG

D- Spirometry

5- Myasthenia graves crisis is characterized by which of the following

A- Sever hypertension

B- Respiratory failure

C- Seizures

D- Hyperglycemia

6- Mention 4 diagnostic test for diagnosis of MG

7- Mention 4 teaching tips for patients with Myasthenia gravis

8- Mention 4 adverse effects and signs of toxicity of anticholinesterase drugs

Seizures

1- A seizure is defined as

A- Permanent brain damage

B- Sudden abnormal electrical activity of the brain

C- Loss of consciousness only

D- Infection of the brain

2- Which of the following is a common cause of seizures

A- Hypertension

B- Fever

C- Hyperlipidemia

D- Arthritis

3- Which examination is most useful in diagnosis of seizure disorder

A- CT scan

B- MRI

C- EEG

D- X-ray

4- Postictal phase refers to

A- Before seizure

B- During seizure

C- After seizure

D- Between seizure

5- Which electrolyte imbalance can cause seizures

A- Hyperkalemia

B- Hyponatremia

C- Hypercalcemia

D- Hypermagnesemia

6- Mention 4 causes of seizures

7- Mention 4 nursing interventions during seizures

8- Mention 4 nursing interventions during postictal period

ICP

1- Which of the following is the earliest sign of increase intracranial pressure

A- Bradycardia

B- Hypertension

C- Disturbed level of consciousness

D- Fixed dilated pupil

2- Cushing triad includes all except

A- Hypertension

B- Bradycardia

C- Irregular respiration

D- Tachycardia

3- Which position is best to help reduce ICP

a- Supine flat position

b- Head elevation 30 degree position

c- Trendelenburg position

d- Prone position

4- Which of the following medication is commonly used to reduce ICP

A- Mannitol

B- Aspirin

C- Metformin

D- Atropine

5- Normal intracranial pressure in adult is

A- 0-5

B- 5-15

C- 0-15

D- 20-30

6- Which of the following can increase ICP

A- hyperventilation

B- hypoventilation

C- sedation

D- hypotension

- 7- which diagnostic tool is most effective I assessment of ICP
A- MRI
B- CT
C- Intrajugular catheter measurement cerebral oxygenation
D- Lumbar puncture
- 8- Mention 4 diagnostics test to determine increase ICP
- 9- Mention 4 nursing percussion for patient with increase ICP
- 10- Mention 4 medical management to patients with increase ICP

Craniotomy

- 1- What is the define of craniotomy
A- Removal of brain tissue
B- Surgical opening of the skull with replacement of bone flap
C- Permanent removal of skull bone
D- Drainage of cerebrospinal fluid
- 2- Which of the following is an indication for craniotomy
A- Migraine
B- Brain tumor
C- Hypertension
D- Diabetes mellitus
- 3- Removal of a bone flap without replacement is called
A- Craniotomy
B- Craniectomy
C- Lobectomy
D- Laminectomy
- 4- Which of the following is a major complication after craniotomy is
A- Hypoglycemia
B- Infection
C- Asthma
D- Peptic ulcer

- 5- A patient Develops fluid leakage from nose after craniotomy this indicate which of the following ?
- a- infection
 - b- CSF leakage**
 - c- Allergy
 - d- Sinusitis
- 6- Which of the following is a goal of nursing care after craniotomy ?
- A- Increase ICP
 - B- Prevent complication**
 - C- Promote hyperventilation
 - D- Restrict extra fluid
- 7- Mention 4 indication for craniotomy
- 8- Mention 4 pre operative care for critically ill patients before craniotomy
- 9- Mention 4 complication after craniotomy

Bacterial meningitis

- 1- Which of the following medical condition consider risk factors for bacterial meningitis
- A- Asthma
 - B- Chronic kidney disease
 - C- Cerebrospinal fluid leak**
 - D- Peptic ulcer
- 2- Which of the following preventive factor reduces risk of bacterial meningitis
- A- Smoking
 - B- Overcrowding
 - C- Vaccination**
 - D- Poor hygiene
- 3- In bacterial meningitis, CSF , glucose is usually
- A- Increased
 - B- Decreased**
 - C- Normal
 - D- Absent

- 4- Which test help to identify the causative organism rapidly in bacterial meningitis
A- Gram strain culture
B- ECG
C- Urine analysis
D- CBC
- 5- Which neurological symptoms may occur in bacterial meningitis
A- Hyperactivity
B- Hearing impairment
C- Seizures
D- Fatigue
- 6- Which of following systemic complication developed with bacterial meningitis
A- Hypertension
B- Seizures and hemiparesis
C- Hyperthyroidism
D- Cataract

Mention 4 nursing diagnosis for patients with bacterial meningitis

Mention 4 nursing intervention for patients with bacterial meningitis

Mention 4 complication for patients with bacterial meningitis

Thyroid crisis

MCQ Thyroid Crisis

The patient with thyroid storm is considered:

- A) Mildly ill and requires minimal care
- B) Critically ill and requires intensive monitoring and care
- C) Stable and does not need observation
- D) Only needs medication without nursing care

Question 1:

Which of the following are iodine-dependent hormones?

- A) Calcitonin and insulin
- B) Thyroxine (T4) and Triiodothyronine (T3)
- C) T3 and calcitonin
- D) Insulin and glucagon

Answer:

- B) Thyroxine (T4) and Triiodothyronine (T3)

Question 2:

Thyroxine is also known as:

- A) T3
- B) T4
- C) T2
- D) T1

Answer:

- B) T4

Question 3:

Triiodothyronine is abbreviated as:

- A) T1
- B) T2
- C) T3
- D) T4

Answer:

- C) T3

Question 4:

Calcitonin is produced by:

- A) Follicular cells
- B) Parathyroid glands
- C) C cells of the thyroid gland
- D) Pituitary gland

Answer:

- C) C cells of the thyroid gland

Question 5:

Calcitonin helps regulate:

- A) Blood sugar levels
- B) Calcium levels in the blood
- C) Sodium levels
- D) Potassium levels

Answer:

- B) Calcium levels in the blood

Question 6

Thyroid hormones primarily regulate:

- A) Blood clotting
- B) Metabolic processes and growth rate
- C) Immune response
- D) Kidney filtration

Answer:

- B) Metabolic processes and growth rate

Question 7:

Thyroid hormones increase:

- A) Oxygen consumption
- B) Carbon dioxide removal only
- C) Water retention
- D) Sodium loss

Answer:

- A) Oxygen consumption

Question 8:

The effect of thyroid hormones on the heart includes:

- A) Decreasing contractility
- B) No effect on the heart
- C) Increasing contractility
- D) Causing heart block

Answer:

- C) Increasing contractility

Question 9:

Thyroid hormones play a role in:

- A) Iron absorption
- B) Calcium absorption
- C) Glucose excretion
- D) Protein loss

Answer:

- B) Calcium absorption

Question 10:

Thyroid storm is also known as:

- A) Hypothyroid crisis
- B) Myxedema coma
- C) Thyrotoxic crisis
- D) Addisonian crisis

Answer:

- C) Thyrotoxic crisis

Question 2:

Thyroid storm is a form of:

- A) Hypothyroidism
- B) Severe hyperthyroidism
- C) Normal thyroid function
- D) Parathyroid disorder

Answer:

- B) Severe hyperthyroidism

Liver transplantation

Which of the following is considered an absolute contraindication for undergoing a liver transplantation?

- A. Sepsis
- B. Primary biliary cirrhosis
- C. History of Hepatitis C infection
- D. Age over 40 years

Which set of laboratory values is used to determine an adult patient's priority on the liver transplant waiting list through the MELD score?

- A. AST, ALT, and alkaline phosphatase
- B. Platelet count, , and glucose
- C. Albumin, ammonia, and lactate
- D. Creatinine, bilirubin, and INR

2- A patient with Wilson's disease is being evaluated for a transplant. What is the underlying cause of liver failure in this condition?

- A. The immune system inappropriately attacking the bile ducts
- B. The accumulation of abnormal amyloid proteins in liver tissue
- C. Abnormal deposition of copper due to impaired hepatic excretion
- D. Excessive absorption and storage of iron in the intestine

Liver dialysis

Which substance is identified as a marker for the accumulation of albumin-bound substances in intrahepatic cholestasis?

- A. Ammonia
- B. Urea
- C. Bilirubin
- D. Creatinine

Which of the following is categorized as a 'biological' liver dialysis system?

- A. Single Pass Albumin Dialysis (SPAD)
- B. Prometheus System
- C. Extracorporeal Liver Assist Device (ELAD)
- D. Molecular Adsorbent Recirculating System (MARS)

28- Why is the choice of anticoagulation regimen particularly 'brittle' in liver failure patients undergoing dialysis?

- A. Heparin is metabolized too quickly by the failing liver, requiring massive doses.
- B. The patient has a high risk of systemic bleeding while the extracorporeal circuit risks triggering clotting.
- C. Anticoagulants interfere with the binding of bilirubin to albumin in the dialysis circuit.
- D. Liver failure patients produce too much heparin naturally, making external dosing difficult.

29- How does Single Pass Albumin Dialysis (SPAD) differ from the MARS system regarding the dialysate?

- A. SPAD uses a recirculating circuit, whereas MARS discards the albumin after one pass.
- B. SPAD incorporates live pig liver cells into the dialysate flow.
- C. SPAD uses a albumin solution, while MARS uses a solution.
- D. SPAD discards the albumin solution after it passes the filter once.

30- What role does glutamine play in the development of brain edema during liver failure?

- A. It acts as a vasodilator that lowers mean arterial pressure.
- B. It accumulates as a metabolic product of ammonia, causing astrocytes to swell.
- C. Glutamine prevents the absorption of benzodiazepines, leading to coma.
- D. Glutamine is a toxin removed by the charcoal filter to prevent cardiac arrest.

What is the typical duration and frequency of a liver dialysis treatment session?

- A. 24 hours a day for up to two weeks continuously.
- B. 4-6 hours per day for 2-5 consecutive days.
- C. A single 12-hour session that completely clears all hepatic toxins.
- D. Once a week for 8 hours until a transplant is found.

Nutrition

A patient on tube feeding develops a sudden onset of fever, tachycardia, and a new pulmonary infiltrate on X-ray. What complication is most likely?

- A. Refeeding syndrome
- B. Tube occlusion
- C. Gastrointestinal dysmotility
- D. Pulmonary aspiration

- During the insertion of a Central Venous Catheter, why is the patient asked to perform the Valsalva maneuver?

- A. To help the catheter move toward the heart
- B. To prevent hypoglycemia
- C. To prevent air embolism
- D. To reduce the risk of phlebitis

Which position is recommended for a patient during the insertion of a subclavian central venous catheter to reduce complications?

- A. Left lateral decubitus
- B. Prone position
- C. High Fowler's position
- D. Trendelenburg position

Fat emulsions are typically administered how often to provide essential fatty acids to a patient on PN?

- A. Daily with every bag of TPN
- B. Only once a month
- C. Every 4 as a bolus
- D. Twice weekly

Hepatic Encephalopathy

Which two major alterations are primarily responsible for the development of hepatic encephalopathy in patients with liver disease?

- A. Biliary obstruction and gallbladder inflammation
- B. Hepatic insufficiency and portal systemic shunting
- C. Bacterial infection and esophageal varices
- D. Hypokalemia and increased renal tubular fluid

A patient is displaying a 'flapping tremor' of the hands. In which stage of hepatic encephalopathy is this clinical manifestation typically first observed?

- A. Stage Two
- B. Stage Four
- C. Stage One
- D. Stage Three

77- Which specific diagnostic finding is characteristic of the electroencephalogram (EEG) in a patient with hepatic encephalopathy?

- A. Alpha rhythm of 8 to 12 Hz in the occipital region
- B. Rapid, low-voltage beta activity
- C. Generalized slowing and triphasic waves
- D. Localized spikes in the temporal lobe

78- How does the administration of Lactulose help reduce serum ammonia levels?

- A. It increases the absorption of protein in the small intestine.
- B. It lowers the colon pH to trap ammonia in an ionized state.
- C. It directly binds to ammonia in the blood and filters it through the kidneys.
- D. It stimulates the liver to produce more urea cycle enzymes.

DKA

Which condition is identified as a major precipitating factor that causes diabetic patients to progress into DKA?

- A. Excessive exercise without dietary adjustment.
- B. Acute infection.
- C. High protein intake.
- D. Over-hydration with hypotonic fluids.

When managing DKA, at what blood glucose level should the IV solution be changed to include 5% Dextrose?

- A. 300 mg/dL
- B. 100 mg/dL
- C. 500 mg/dL
- D. 200 mg/dL

What is the recommended hourly rate for the decline of plasma glucose levels when administering an insulin infusion for DKA?

- A. 5% to 10% of the total glucose per hour
- B. 150 to 200 mg/dL per hour
- C. 50 to 75 mg/dL per hour
- D. 10 to 25 mg/dL per hour

- A nurse is caring for a 70 kg patient with DKA. According to the standard insulin infusion protocol mentioned, what would be the initial hourly infusion rate of regular insulin?

- A. 7 units per hour
- B. 1 unit per hour
- C. 70 units per hour
- D. 14 units per hour

Which non-diabetic condition is listed as a potential cause for the development of DKA?

- A. High-carbohydrate dieting
- B. Excessive vitamin intake
- C. Hypothyroidism
- D. Starvation